



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : ساميه صبحى جيد عبيد
المحافظة : القاهرة الحي / المدينة : حلوان
رقم البطاقة : 27401190104008 رقم التليفون : 01204269082
العمر (تاريخ الميلاد) : 19-01-1974 المهنة : ربه منزل
العنوان : الهرم الجيزة
تاريخ تسجيل الإستمارة : 30-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي :
التوقيع : (-----)
المقرر بما فيه :
التوقيع : (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر والحوض
بالصبغة [11127]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ساميه صبحي جيد عبد National ID: 27401190104008	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تخليق الشخصية

ساميه
صبيح جند عبد
٨٠ من مصطفى رفاعي الضابط البحري
الضابط - البحري

٢٧٤٠١١٩٠١٠٤٠٠٨

IM4269031

٢٠٢٠/٠٧ ٢٧٤٠١١٩٠١٠٤٠٠٨
حاصله على دبلوم تجارة

التي مسيحية أرمنية

البطاقة سارية حتى ٢٠٢٧/٠٧/٢١



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

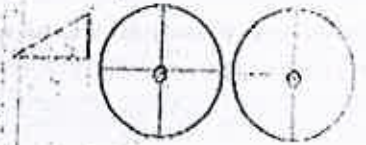
1- Name : منال محمد جبريل

MDT form

3- Medical numbers
Female03174059
Old Med :CP Med :
4394-2023

2- ID numbers 27401190104008

Patient Age : 50 Y



Family history

Family History

Phone

Phone

BMI

BMI

REASON OF MDT

REASON OF MDT
POST OPERATIVE

marital status

Marital Status

HRS Score

HRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co morbidity

Co Morbidity

Menopausal
statusMenopausal Status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE
Early DCIS

31/12/2023

Breast Imaging :

Clinical data

Mets Workup :
ct cap: free

BX

ER

PR

Her 2 Neu

SISH

IDC

4/8

0/8

3

SIDE COMMENT :

BX COMMENT:
+DCIS

BX COMMENT :

BX COMMENT :

Her 2 Neu COMMENT :

T
T1mlcN-AP
N 1mlcM
0

Sonomammography

PATIENT HISTORY

It mastectomy: free margins , IDC , G.II , with high grade DCIS.
T1mlc, paget's N 1mlc/24

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

For :

- 6 cycles of adjuvant TC + 9 cycles single anti-HER 2 agent Herceptin.
- Then Advant Tamoxifen for 5 years.

MEDICAL ONCOLOGY

SIGNATURE

SURGERY

Psychiatric

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

PATHOLOGY

Nutrition

RADIOLOGY

RADIOLOGY

RADIOLOGY

RADIOLOGY

RADIOLOGY

RADIOLOGY

Name: ساميه صبحي جيد			
MR. : 3796			
Detailed Medical Report			
		السيد	
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الخروج :		تاريخ وساعة الدخول :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 52-year-old female with a history of type 2 diabetes mellitus who underwent an upfront left mastectomy in October 2023. The surgical specimen demonstrated invasive ductal carcinoma, grade II, with high-grade ductal features, clear surgical margins, and a tumor size of 4 cm. Immunohistochemistry revealed ER 4/8, PR 0/8, and HER-2 3+. Post-mastectomy adjuvant therapy consisted of six cycles of docetaxel-cyclophosphamide (TC) followed by nine cycles of single-agent trastuzumab (Herceptin). The patient subsequently continued adjuvant tamoxifen therapy beginning March 2024 for a duration of three months. The patient has completed 6 cycles of TC and 9 cycles of Herceptin.			
تحريرا في ,			
التاريخ :		20-09-2026	
الوقت :			
علياء احمد أبو النصر شهاب الدين		الطبيب المعالج :	
يعتمد ,			

Patient Name : السيد / سامية صبحي جود عبد الله
Patient No. : 27401190104008
Sh No. : SR-1454/23
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 13006024
Request Date : 25/11/2023
Verification Date : 09/12/2023
Financial Category : أورام دار السلام - نفقة الدولة
Department : Out Patient
Registered by : Ali mostafa

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide Revision

Gross Examination:

Thirteen paraffin blocks labeled by (7728/23), one paraffin block labeled by (7035/23) and three slides were received.

Microscopic Examination:

Histological type: Invasive duct carcinoma

Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1).

DCIS: Wide, High grade.

Skin involvement: Ulcerated skin infiltrated with tumor cells.

Lymph node involvement: Showed micrometastasis in one nodule.

IPT (One lymph node):

ER: Weak nuclear reaction in 5% of tumor cells (Allred score 3/8).

PR: Weak nuclear reaction in 5% of tumor cells (Allred score 3/8).

Her2: Strong membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:

Left breast cancer, Slide revision:

Invasive duct carcinoma, Grade II, with wide, high grade DCIS

Skin infiltrated.

One nodule shows micrometastasis

IPT:

ER: Positive (score 3/8).

PR: Positive (score 3/8).

Her2: Positive (score 3+).

Verified by :

Dr. Marwa mahmoud

Printing Date : 20/12/2023 11:12:37

Page 1 of 2

Printing By : marwa mohamed youss



شركة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadima - Cairo, Egypt

Patient Name : الأستاذة / سامية صبحي جند صيد عبده

Patient No. : 27401190104008

Ref No. : SR-1454/23

Sex : Female

Age : 49 Y

Referring Doc :

Request No. : 13006024

Request Date : 25/11/2023

Verification Date : 09/12/2023

Financial Category : أورام دار السلام - نفقة الدولة

Department : Out Patient

Registered by : Ali mostafa

Pathology Report

Signature

Dr. Rana Fathy

Dr. Menan Hossam

Verified by :

Dr. Marwa mahmoud

Printing Date : 20/12/2023 11:12:37

Page 2 of 2

Printing By : marwa mohamed youss

CAP
RECEIVED
12/20/2023غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadima - Cairo, Egypt



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : سامية صبحي جبد عبيد

Sex:

3- Medical number:
Female

03174059

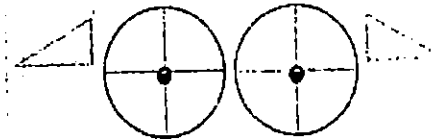
Old Med :

Old Med :

4394-2023

2-ID number: 27401190104008

Patient Age : 50 Y



Family history

Family History

Phone

Phone

01225997466

BMI

BMI

REASON OF MDT

REASON OF MDT
POST OPERATIVE

marital status

Marital Status
WIDOW

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

31/12/2023 ☐

Co morbidity

Co Morbidity

Menopausal
statusMenopausal Status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE
Early Local

Clinical data

Breast Imaging :

Mets Workup :
ct cap: freeBX
IDCER
4/8PR
0/8Her 2 Neu
3

SISH

Ki67

BX COMMENT:
+DCIS

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT:

T
T1micN
N 1micM
0

Sonomammography

PATIENT HISTORY

It mastectomy: free margins , IDC , G.II , with high grade DCIS.
T1mic, paget's N 1mic/24



ERM036724465

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

For :

- 6 cycles of adjuvant TC + 9 cycles single anti-HER 2 agent Herceptin.
- Then Adjvant Tamoxifen for 5 years.

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ داليا ابراهيم د/ كاترين ناصيف	RADIOLOGY د/ د. هادي	RADIOLOGY د/ د. هادي
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Echo report

Name:

Date: 3/9/2024

MR: ٤٢٩٤
٢٢

EF	69%	55-75%	PWT	0.7	0.6-1.1cm
LVEDD	4.4	3.5-5.8cm	AO.R	2.4	2-3.7cm
LVESD		2-4cm	LA	3	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LVDimensions

normal

LV wall thickness

normal

LV diastolic function

normal

SWMA

normal

LA and Aortic root dimensions

normal

Mitral Valve

normal

Aortic valve

normal

Tricuspid valve

Trivial TR

Pulmonary valve

normal

Right side and systolic function

normal

Intracardiac masses and effusion

normal

D/Msc cardiology

Dr. [Signature]



Echo report

MR ٢٩٤
C.٣

Name: محمد علي
Date: 6/6/2024

EF	68%	55-75%	PWT		0.6-1.1cm
LVEDD	4.3	3.5-5.8cm	AO.R	3.3	2-3.7cm
LVESD	2.7	2-4cm	LA	3.4	2-4cm
FS	38%	25-45%	RV		0.7-2.3cm(supine)
SWT	1.1	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions

LV wall thickness

LV diastolic function

SWMA No SWMA at rest

LA and Aortic root dimensions

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion

NO

D/Mohamed Ali

Msc cardiology



وزارة الصحة والسكان
امانة المستشفى الطبية المتخصصة
مستشفى اورام دار السلام

Echo report

Name:

Date: // // 2024

MR ٤١٩٤
٢٢

EF	67%	55-75%	PWT		0.6-1.1cm
LVEDD	3.6	3.5-5.8cm	AO.R	3.0	2-3.7cm
LVESD	2.4	2-4cm	LA	3.5	2-4cm
FS	39%	25-45%	RV		0.7-2.3cm(supine)
SWT	1.07	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions

LV wall thickness

LV diastolic function

SWMA No RWMA at rest

LA and Aortic root dimensions

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion

D/ Mohamed Ali

Msc cardiology



Patient Name	: الأستاذ / ساميه صبحي جيد عبيد عبده	Request No.	: 13006024
Patient No.	: 27401190104008	Request Date	: 25/11/2023
Path No.	: SR-1454/23	Verification Date	: 09/12/2023
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 49 Y	Department	: Out Patient
Referring Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide Revision

Gross Examination:

- Thirteen paraffin blocks labeled by (7728/23), one paraffin block labeled by (7035/23) and Three slides were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1).
- * DCIS: Wide, High grade.
- * Skin involvement: Ulcerated skin infiltrated with tumor cells.
- * Lymph node involvement: Showed micrometastasis in one nodule.
- * IPT (One lymph node):
- ER: Weak nuclear reaction in 5% of tumor cells (Allred score 3/8).
- PR: Weak nuclear reaction in 5% of tumor cells (Allred score 3/8).
- Her2: Strong membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:

Left breast cancer, Slide revision:

Invasive duct carcinoma, Grade II, with wide, high grade DCIS

- Skin infiltrated.
- One nodule shows micrometastasis
- * IPT:
- ER: Positive (score 3/8).
- PR: Positive (score 3/8).
- Her2: Positive (score 3+).

Verified by :

Dr.Marwa mahmoud

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Page 1 of 2

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Patient Name	: الأستاذ / ساميه صبحي جيد عبيد عبده	Request No.	: 13006024
Patient No.	: 27401190104008	Request Date	: 25/11/2023
Path No.	: SR-1454/23	Verification Date	: 09/12/2023
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 49 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Signature
Dr. Rana Fathy
Dr. Menan Hossam

قسم الباثولوجي

Verified by :
Dr.Marwa mahmoud

Printing Date : 20/12/2023 11:12:37

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Date : 24/09/2024	Patient Name: Samia Sobhy Gayed Ebeid Abdou
Age : 50 Year(s)	Exam : ultrasound TRANS VAGINAL
Pat. ID: 79425	Medical ID: 4394/2023

Trans-vaginal ultrasound scanning revealed: -

○ **Uterus: -**

- *AVF, average sized uterus measuring about: 6 x 4 x 3.7 cm in its longitudinal, transverse & AP diameters respectively.*
- *Endometrial thickness measures about: 6 mm.*
- *No gross myometrial focal lesions.*

○ **Ovaries: -**

- *Both ovaries are of average size, normal follicular pattern with **no** dominant follicle could be detected.*
- *Right ovary measures about: 2.6 x 2.1 cm.*
- *Left ovary measures about: 2.6 x 2.1 cm.*

- *No gross adnexal soft tissue masses or cysts.*
- *No free fluid in the Douglas pouch.*

Conclusion:

Normal study

*Best regards
Dr/Dina Gaber ,FEBR*

Not Approved

MRN: 2022197848
Patient Name: SAM'A SOBHY GAED EBEED

D.O.B: 19/01/1974
IC No.: 27401190104008
Sex: Female
Location: External Clinic

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id 2510290483
Accession No U/M23-6380
Sample Type Paraffin Block

Collection Date 29/10/2023 08:13 PM
Received Date 29/10/2023 08:13 PM
Doctor Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number

23-7728

Clinical Data

Lt. breast DCIS with microinvasion.

Metastatic LN.

Gross Picture :

Referred block submitted for ER/PR Immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :

According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak. 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:

- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

ER positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:

left

Diagnosis :

ER: Positive, weak nuclear reaction in 20% of tumor cells [4/8].

PR: Negative [0/8].

Signature

Dr. Dina Mohamed Sweed

Co-Signed by:

Heba Abdallah

Reported and electronically endorsed by
dina mohamed mostafa sawed

Calcification:

Yes

Resection margins

Invasive carcinoma at resection margin:

No

DCIS at resection margin:

No

Paget's disease of nipple

Present

Axillary lymph node dissection

Nature of nodal involvement :

Focal

Diagnosis

1- Lt. breast. Modified radical mastectomy:

High grade ductal carcinoma insitu with focal stromal microinvasion, pT(mi)N1mi

Resection margins: Free.

Axillary LNs: Positive for micrometastasis [1/24] with no perinodal fat infiltration.

2- Extra deep margin: Free of tumor.

Comments

ER, PR and Her2 are ordered on block J

microinvasion Block Q

re-serial...1/11/2023

Category

Malignant primary site

Signature

DR.Nesma Mohamed Salah Elden

Co-signed by:

DR.Dina Mohamed Sweed

Reported and electronically endorsed by
dina mohamed mostafa sawed

MRN: 2022197848
Patient Name: SAMIA SOBHY GAED EBEED

D.O.B: 19/01/1974
IC No.: 27401190104008
Sex: Female
Location: External Clinic

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 2310290484
Accession No IMM23-6381
Sample Type Paraffin Block

Collection Date 29/10/2023 06:13 PM
Received Date 29/10/2023 06:13 PM
Doctor Abd ALLAH Mohamed Gamil

Immunohistochemical Report

pathology number
23-7728

Clinical Data

Lt. Breast DCIS with microinvasion.

Metastatic LN.

Gross Picture :

Referred block submitted for HER 2 Immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :

Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within <=10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within >10% of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in >10% of tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense in >10 % of tumor cells.

(Laterality) breast Cancer:

left

Laterality - 1

Left

Her2 Result - 1

Positive [score 3+]

Signature

Dr.Dina Mohamed Sweed

Co-Signed by:

Reported and electronically endorsed by
dina mohamed mostafa sawed

MRN: 2022197646
Patient Name: SAMIA SOBHY GAED EBED

D.O.B: 19/1/1974
IC No.: 27431190104008
Sex: Female
Location: External Clinic

SURGICAL PATHOLOGY

Specimen examination - (1 Site)

Specimen Id	2310170027	Collection Date	17/10/2023 09:08 AM
Accession No	SUP23-7728	Received Date	17/10/2023 09:08 AM
Sample Type	Tissue	Doctor	Abd ALLAH Mohamed Gamil

Clinical Information

Left breast mass diagnosed as high grade DCIS on a biopsy NO. 23-7035 .

Gross Picture

Two specimens were received formalin fixed:

1- Left modified radical mastectomy specimen (19x15cm) with a skin ellipse (17x18cm) that included an attached nipple and an attached axillary tail (7x5x2.5cm) entangling multiple lymph nodes, largest one is 1x1cm. Serial sections revealed a lower inner quadrant **nodular area (6.5x6 cm)**, located 0.5 cm from the skin and 0.2 cm from the deep margin.

2- **Extra-deep margin:** fibrofatty tissue, totally submitted.

Microscopic Picture

1- A case of ductal carcinoma insitu, intermediate to high grade of cribriform and solid pattern with comedo necrosis. There is focal stromal microinvasion. Adjacent breast shows fibrocystic disease with apocrine metaplasia and calcifications. Nipple shows **Paget's disease**. **Deep margin is free of tumor cells**. Skin margins are free of tumor. One axillary lymph node is **positive for micrometastasis (2/24)** with no perinodal infiltration.

2- **Extra deep margin:** free of tumor. ❌

Diagnostic Summary

Tumour type:

High grade ductal carcinoma insitu with focal stromal microinvasion.

Specimen type:

Modified radical mastectomy.

Laterality

Left

Location

Lesion Size

Invasive tumour size/s (mm):

Tumour (cm) (mm)

Metastatic grade

Metastatic site

Metastatic

Metastatic site

Nuclear grade:

High

Necrosis:

Yes

Architecture:

Cribriform



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : سلمية صبحي جريد عبيد

Sex:

3- Medical number:

Female

83174059

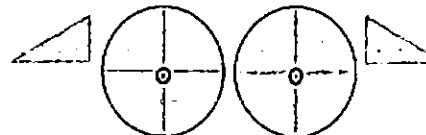
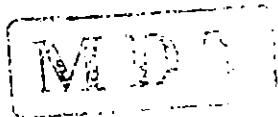
Old Med :

Old Med :

4394-2023

2-ID number: 27401190104008

Patient Age : 50 Y



Family history

Family History

Phone

Phone

01225997466

BMI

BMI

REASON OF MDT

REASON OF MDT
POST OPERATIVE

marital status

Marital Status
WIDOW

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

31/12/2023

Co morbidity

Co Morbidity

Menopausal
statusMenopausal Status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE
Early Detect

Clinical data

Breast Imaging :

Mets Workup :

ct cap: free

BX IDC BX COMMENT: +DCIS	ER 4/8 ER COMMENT:	PR 0/8 PR COMMENT:	Her 2 Neu 3 Her 2 Neu COMMENT:	SISH SISH COMMENT :	KI67 KI67 COMMENT :
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T T1mic	N N 1mic	M 0
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Sonomammography

PATIENT HISTORY

It mastectomy: free margins , IDC , G.II , with high grade DCIS.
T1mic, paget's N 1mic/24

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

For :

- 6 cycles of adjuvant TC + 9 cycles single anti-HER 2 agent Hereceptin.
- Then Adjvant Tamoxifen for 5 years.

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/ داليا ابراهيم د/ مكارم نصيف	RADIOLOGY	RADIOLOGY د/ امل حجازي
SURGERY	SURGERY د/ مكارم نصيف	PATHOLOGY	PATHOLOGY د/ مكارم نصيف
Psychiatric	Psychiatric	Nutrition	Nutrition